

HEALTHCONNECT CLINIC EXPERIENCE LAB

UPDATED RISK & DEPENDENCY REGISTER

Improving Patient Appointment Attendance and Healthcare Support Using Data and AI

Document Control

Project	HealthConnect Clinic Experience Lab
Project Title	Improving Patient Appointment Attendance and Healthcare Support Using Data and AI
Document Type	Updated Risk & Dependency Register
Version	2.0
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Project Phase	Solution Development & Implementation
Register Owner	Project Management Function
Review Authority	Project Sponsor / Project Leadership
Status	Active Monitoring

1. PURPOSE AND CONTROL OBJECTIVE

This register provides the controlled project-level record of risks and dependencies that may affect delivery of the HealthConnect Clinic Experience Lab.

It supports four management functions:

1. Identifying emerging threats before they become delivery issues;
2. Assigning ownership for active risk treatment and dependency management;
3. Monitoring conditions that may affect downstream workstreams;
4. Providing an escalation path where exposure exceeds workstream-level control.

The register is maintained as a project control document. It does not replace the Task Tracker, Project Status Report, Issue Log or Schedule. Those controls retain their respective purposes.

2. RISK MANAGEMENT FRAMEWORK

2.1 Risk Definition

A risk is an uncertain event or condition that may affect project objectives if it occurs.

Risk statements are maintained using the following logic where applicable:

Cause / Condition → Risk Event → Potential Project Effect

2.2 Risk Rating

Risk Score = Probability × Impact

Score	Rating	Control Requirement
1–4	Low	Routine monitoring.
5–9	Moderate	Active owner monitoring.
10–16	High	Defined response and management attention.
17–25	Critical	Immediate escalation and corrective action.

2.3 Probability

Score	Rating
1	Rare
2	Unlikely
3	Possible
4	Likely
5	Almost Certain

2.4 Impact

Impact considers potential effects on delivery, quality, integration, operational suitability, governance and achievement of project objectives.

Score	Rating
1	Insignificant
2	Minor
3	Moderate
4	Major
5	Severe

3. UPDATED RISK REGISTER

Each risk's Area and Owner keep the workstream colour used across the HealthConnect project documents.

Project Management	Data & Analytics	Predictive Intelligence	Patient Engagement	Generative AI Support	Process Integration	Testing & Validation
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ID	Risk Statement	Area	P	I	Exposure	Owner	Preventive / Mitigation Control	Response / Contingency	Trigger / Early Warning	Status
R-01	Data quality limitations may reduce the reliability of attendance analysis and downstream predictive outputs.	Data & Analytics	4	4	High	Data & Analytics Lead	Assess completeness, consistency and analytical suitability before using data for decision-making.	Remediate material limitations where feasible; restrict affected variables or revise analytical approach where necessary.	Material missingness, inconsistent records or unsuitable variables identified.	Open – Monitoring
R-02	Insufficiently validated attendance findings may direct downstream solution development toward inappropriate priorities.	Analytics / Solution Design	3	4	High	Data & Analytics Lead	Validate material findings before they are adopted as inputs to downstream solution design.	Reanalyse affected findings and reassess dependent solution decisions.	Findings lack sufficient evidence or cannot be confidently interpreted.	Open – Monitoring
R-03	Delay in analytical outputs may constrain predictive intelligence and patient-engagement activities.	Schedule / Integration	3	4	High	Project Management Lead	Monitor analytical deliverables against dependent activities and identify handoff requirements early.	Re-sequence dependent activities where feasible and escalate material schedule pressure.	Required analytical output unavailable when a dependent activity is ready to proceed.	Open – Monitoring
R-04	Predictive development may produce a technically acceptable model that provides insufficient value for operational decision-making.	Predictive Intelligence	3	4	High	AI / ML Lead	Evaluate the predictive approach against defined technical and operational criteria rather than accuracy alone.	Refine the model, variables or intended use case where evaluation criteria are not met.	Model fails agreed evaluation criteria or provides limited actionable value.	Open – Monitoring
R-05	Patient engagement interventions may not sufficiently reflect observed attendance behaviour.	Patient Engagement	3	3	Moderate	Patient Engagement Lead	Link intervention design to validated attendance patterns and identified patient-support needs.	Revise intervention logic and revalidate against project evidence.	Proposed intervention cannot be clearly connected to an identified attendance behaviour or barrier.	Open – Monitoring
R-06	Generative AI support may exceed approved use boundaries or lack adequate escalation controls.	Generative AI Support	3	5	High	GenAI Lead / Responsible AI Oversight	Define approved use cases, knowledge boundaries, response constraints and escalation requirements.	Restrict affected functionality, strengthen controls or remove unsupported use cases.	Unsupported responses, unclear escalation routes or use outside approved boundaries.	Open – Monitoring
R-07	Workstream outputs may be developed without sufficient alignment, creating integration rework.	Cross-Workstream Integration	3	4	High	Project Management Lead	Maintain defined interfaces, handoff criteria, ownership and coordination checkpoints.	Resolve interface conflicts and re-coordinate affected outputs before downstream use.	Output changes affect another workstream without corresponding coordination.	Open – Active Control
R-08	Operational workflow	Process &	3	4	High	Process &	Validate proposed capabilities	Redesign affected	Proposed capability	Open –

ID	Risk Statement	Area	P	I	Exposure	Owner	Preventive / Mitigation Control	Response / Contingency	Trigger / Early Warning	Status
	requirements may not be adequately reflected in solution design.	Operational Integration				Operational Integration Lead	against clinic processes, roles and operational requirements.	workflow or solution component before validation.	cannot be clearly mapped to an operational process or responsibility.	Monitoring
R-09	Validation requirements identified too late may create avoidable redesign and delivery pressure.	Testing & Validation	3	4	High	Testing & Validation Lead	Establish validation requirements while solution components are being developed.	Prioritise critical acceptance requirements and re-sequence lower-priority validation activity.	New acceptance requirements emerge after development has materially progressed.	Open – Monitoring
R-10	Emerging execution constraints may remain unidentified until they affect downstream activities.	Project Execution	3	4	High	Project Management Lead	Use regular progress reviews, exception monitoring and workstream reporting to identify emerging constraints.	Escalate confirmed constraints and initiate corrective action or recovery planning.	Repeated slippage, unresolved blockers or unexplained progress deterioration.	Open – Active Control
R-11	Stakeholder or operational expectations may diverge from emerging technical outputs.	Stakeholder / Solution Alignment	2	4	Moderate	Project Management / Workstream Leads	Review material outputs against agreed requirements and operational expectations.	Reconcile requirements and apply controlled changes where necessary.	Stakeholder review identifies material mismatch between expected and emerging capability.	Open – Monitoring
R-12	Increased parallel activity may create coordination overhead or conflicting priorities across workstreams.	Programme Coordination	3	3	Moderate	Project Management Lead	Maintain integrated priorities, dependency visibility and defined coordination checkpoints.	Re-prioritise activities and resolve competing requirements through project coordination.	Multiple workstreams require the same input, decision or resource within overlapping periods.	Open – Active Control

4. RISK MOVEMENT

The transition from planning into active analysis and solution development has changed the nature of project exposure, particularly around interfaces between workstreams.

The principal movements are:

Data and Analytical Exposure

Data quality has become more consequential because analytical outputs are now being used to inform subsequent project decisions. This makes data limitations an active delivery concern rather than solely a technical consideration.

Downstream Dependency Exposure

Attendance analysis now represents an important upstream input to predictive intelligence and patient-engagement activities. Delays or weaknesses at this point may therefore propagate across multiple workstreams.

Integration Exposure

Parallel development has increased the requirement for explicit handoffs and interface control. A technically complete output is not considered fully effective until the receiving workstream can use it for its intended purpose.

Validation Exposure

As solution components mature, late identification of acceptance requirements could create avoidable rework. Validation requirements therefore require visibility during development.

Current risk position: no Critical risk identified at the status date. High risks remain subject to active mitigation and monitoring.

5. DEPENDENCY MANAGEMENT FRAMEWORK

A dependency exists where the progress or completion of one activity, output, decision or workstream relies upon another activity, output or condition.

Dependencies are distinguished from risks because they describe required relationships, while risks describe uncertainty and potential adverse effects associated with delivery.

6. DEPENDENCY REGISTER

ID	Dependency	Upstream Requirement	Dependent Area	Owner	Required Handoff / Condition	Current Position	Control
D-01	Approved project baseline	Approved project scope, governance and delivery structure	All workstreams	Project Management	Authorised baseline available for execution	Established	Controlled
D-02	Validated problem and requirements	Confirmed problem definition and project requirements	Data and solution development	Project Management / Requirements Owner	Requirements sufficiently defined for execution	Established	Controlled
D-03	Data availability and quality	Required appointment data and quality assessment	Data & Analytics	Data & Analytics Lead	Data assessed for analytical suitability	Active	Monitor
D-04	Analysis-ready data	Prepared and quality-assessed dataset	Attendance Analytics	Data & Analytics Lead	Dataset suitable for analytical use	Active	Monitor
D-05	Attendance findings	Validated attendance patterns and contributing factors	Predictive Intelligence	Data & Analytics Lead	Findings available as an evidence-based modelling input	Active	Monitor
D-06	Attendance insights	Validated attendance behaviour and contributing factors	Patient Engagement	Data & Analytics / Patient Engagement Leads	Findings translated into intervention requirements	Active	Monitor
D-07	Support requirements and approved knowledge	Defined enquiry-support requirements and authorised knowledge	Generative AI Support	GenAI Lead / Responsible AI Oversight	Approved knowledge and boundaries available	Active	Controlled
D-08	Predictive and engagement outputs	Defined solution components	Operational Integration	Relevant Workstream Leads	Components sufficiently defined for workflow mapping	Future / Active	Monitor
D-09	Integrated workflow	Solution components incorporated into an operational workflow	Testing & Validation	Process & Operational Integration Lead	Integrated workflow available for validation	Future	Planned
D-10	Validation results	Technical, functional and user validation	Implementation Readiness	Testing & Validation Lead	Validation evidence available and critical defects addressed or dispositioned	Future	Planned

ID	Dependency	Upstream Requirement	Dependent Area	Owner	Required Handoff / Condition	Current Position	Control
D-11	Defined outcomes and measures	Agreed outcomes and measurement requirements	Monitoring & Evaluation	Project Management / M&E Lead	KPI and monitoring requirements established	Future	Planned
D-12	Cross-workstream handoffs	Timely transfer of outputs, information and decisions	Integrated Delivery	Project Management Lead	Receiving workstream confirms required input is usable	Active	Active Control

7. CRITICAL DEPENDENCY PATH

The principal dependency structure currently requiring active monitoring is:



The Generative AI Support workstream follows a partially parallel path. Its progression is dependent on defined support requirements, approved knowledge and appropriate usage and escalation controls; it does not require completion of predictive modelling as a prerequisite.

This allows parallel development without removing the control points required for safe and coherent integration.

8. DEPENDENCY CONTROL RULES

All material dependencies shall be controlled against the following requirements:

1. **Named ownership** — every material dependency has an accountable owner.
2. **Defined input** — the required upstream output or condition is explicit.
3. **Defined receiving requirement** — the receiving workstream understands what it requires.
4. **Acceptance condition** — completion of an upstream activity does not automatically mean the downstream requirement has been satisfied.
5. **Timing visibility** — time-sensitive dependencies are visible within project scheduling and coordination.
6. **Change assessment** — material changes to an upstream output are assessed for downstream impact.
7. **Failure conversion** — a failed or materially delayed dependency is assessed for risk and / or issue treatment.
8. **Handoff confirmation** — significant cross-workstream transfers require acknowledgement by the receiving owner.

9. RISK RESPONSE FRAMEWORK

Risk responses shall be selected according to the exposure and the degree of control available to the project.

Response	Application
Avoid	Remove the condition or approach creating unacceptable exposure.
Mitigate	Reduce the probability or impact through preventive or corrective controls.

Response	Application
Transfer / Share	Allocate defined exposure to an appropriate party where practical.
Accept	Retain exposure where it remains within approved tolerance and further treatment is disproportionate.
Escalate	Raise exposure to project leadership where it exceeds workstream authority or agreed tolerance.

Responses should address the underlying cause wherever practicable rather than only treating the eventual consequence.

10. ESCALATION FRAMEWORK

Level 1 — Workstream Control

The responsible workstream owner manages risks and dependency constraints that can be resolved using existing authority, resources and agreed requirements.

Level 2 — Project Management Control

Project Management intervenes where the matter affects:

- Multiple workstreams;
- Cross-workstream handoffs;
- Integrated sequencing;
- Project-level schedule exposure;
- Competing priorities;
- Unresolved dependencies; or
- Co-ordinated corrective action.

Level 3 — Project Leadership Escalation

Project leadership consideration is required where the matter:

- Threatens a major project milestone or stage gate;
- Exceeds established risk tolerance;
- Requires material scope, resource or schedule intervention;
- Cannot be resolved within existing authority;
- Creates significant operational or governance implications; or
- Raises material responsible-AI concerns.

11. RISK REVIEW TRIGGERS

A risk shall be reassessed when:

- Probability or impact changes materially;
- A mitigation control fails or proves ineffective;

- A defined trigger occurs;
- An associated dependency becomes delayed or unavailable;
- A new technical or operational constraint emerges;
- A downstream activity becomes materially affected;
- Stakeholder requirements change;
- A stage-gate review identifies additional exposure; or
- The proposed response requires management intervention.

Risk review is therefore trigger-based as well as periodic.

12. RISK CLOSURE CRITERIA

A risk shall only be closed when there is sufficient evidence that:

1. The underlying risk condition has been removed, controlled or is no longer relevant;
2. The required response has been implemented;
3. Material residual exposure has been assessed;
4. Affected dependencies have been reviewed;
5. Downstream implications have been considered;
6. The risk owner confirms the disposition; and
7. The register has been updated accordingly.

Where exposure remains, the risk shall remain open with its residual position updated rather than being closed solely because a related task has been completed.

13. CURRENT MANAGEMENT ATTENTION

The following areas require continued management attention during the current execution period.

13.1 Data Readiness

Data quality remains an important upstream condition for credible attendance analysis and downstream decision-making.

13.2 Analytical Handoffs

Attendance findings must be sufficiently validated before being used as inputs to predictive and intervention design.

13.3 Cross-Workstream Interfaces

Parallel work requires controlled transfer of outputs, decisions and requirements between workstreams.

13.4 Operational Alignment

Technical outputs must remain connected to actual clinic workflows, responsibilities and patient-support needs.

13.5 Validation Readiness

Validation requirements should remain visible throughout development to minimise late-stage redesign.

13.6 Emerging Execution Constraints

The project should continue active monitoring for constraints that were not visible during initial planning and have only become apparent through execution.

14. CURRENT RISK & DEPENDENCY POSITION

● OVERALL EXPOSURE: AMBER — MANAGEABLE WITH ACTIVE CONTROL

The current register identifies no Critical risk or failed project dependency requiring immediate project-level intervention.

The most significant exposure is concentrated around:

- Data readiness and quality;
- Timing and reliability of analytical outputs;
- Downstream reliance on attendance findings;
- Cross-workstream handoffs;
- Operational alignment; and
- Increasing coordination requirements associated with concurrent workstream activity.

The project can continue through the current execution stage provided identified controls remain active and material changes are escalated promptly.

15. CONTROL SUMMARY

Control Area	Current Position
Risk Identification	Active
Risk Assessment	Established
Risk Ownership	Assigned
Risk Response	Defined
Dependency Identification	Established
Dependency Ownership	Assigned
Cross-Workstream Dependency Monitoring	Active
Escalation Mechanism	Established
Review Triggers	Defined

Control Area	Current Position
Closure Criteria	Defined
Management Attention	Required
Overall Exposure	● Amber – Manageable with Active Control

16. VERSION CONTROL

Version	Date	Change Description	Owner
1.0	August 2026	Initial risk and dependency baseline established during HealthConnect project initiation and planning.	Project Management
2.0	September 2026	Updated for Week 5 execution, incorporating current risk exposure, active dependencies, response controls, escalation requirements and management review criteria.	Project Management

Register Governance

The Project Management Function owns the controlled project-level register.

Workstream leads are accountable for managing risks and dependencies within their areas of responsibility. Project Management maintains integrated visibility, monitors cross-workstream effects, coordinates escalation and ensures material changes are reflected in the appropriate project control.

Next Formal Review: Week 6 project review, or earlier following a material risk trigger, dependency change, issue, decision or stage-gate event.

End of Updated Risk & Dependency Register — Version 2.0